

Why Your Census Number Is Not Arbitrary

What Nobody Tells Hospice Sales Reps About the Business — and the People — They Sell For

At some point early in your hospice sales career, someone gave you a number. Eight admissions a month. Twelve. An ADC of 60 by Q3. What it almost certainly did not come with was an explanation of where it came from — or what it actually means for the people your branch exists to serve.

This document exists to provide that explanation. Not because the numbers are not important — they are. But because a rep who understands why the number exists sells with a different kind of conviction. And a rep who understands who depends on that number never needs to be motivated again.

A Spartan Coaching Field Education Document · Branch Profitability Series

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BEFORE THE NUMBERS

The People Behind the Number — Who Your Census Sustains

The census number you were given is not a business metric dressed up as a goal. It is a human requirement. Every patient admitted to your branch's service places an act of profound trust not just in an organization or a Medicare benefit — but in a specific team of specific people. That trust carries weight unlike almost any other in medicine: these are the people a patient and their family will invite into their home during the most vulnerable, most sacred chapter of a human life.

Your census — the number of patients on service at any given time — is what funds that team. It is what pays their salaries every two weeks, covers the medications in their nursing bags, puts fuel in their cars, and keeps the office running so they can focus entirely on the person in front of them. Without sufficient census, that team cannot exist in full. Without that team in full, the patients who trusted your branch do not receive the care they were promised.

"A patient does not just enroll in a benefit. They entrust a team of people with the most intimate chapter of their life — and their death. The census is what makes it possible to honor that trust."

Hospice is not a building patients go to. It is a team that comes to them — in their home, their assisted living room, their nursing facility bed — and wraps around them with expert, compassionate, coordinated care. The RN case manager who calls at 2 a.m. The hospice aide who helps a man bathe with dignity on what may be one of his last mornings. The social worker who sits with a daughter who has been managing medications, navigating insurance, and holding the family together for months — and finally tells her that she does not have to do this alone anymore. The chaplain who shows up without an agenda and simply asks what a person needs to say.

These are not support roles. These are the most influential people in the dying process. They shape what a patient's final weeks feel like. They determine whether a family member looks back on that time with peace or with regret. They are the reason hospice exists — and they are only there, in full, when the branch census is high enough to sustain them.

The number you were given is not a quota. It is the minimum condition under which the most influential people in hospice can actually do their jobs.

PART ONE

The Clinical Team: The Heart of Every Patient's Final Chapter

Every line item in a hospice payroll is a person.

Every line item in a hospice payroll is a person. Not a position. Not a job title. A person with training, credentials, relationships, and the weight of walking into someone's home on one of the worst days of their family's life — and making it better. Understanding your branch's cost structure begins with understanding these people and what they actually do for the patients your census supports.

The Clinical Team

These roles are not optional. Medicare Conditions of Participation require every certified hospice to maintain this team regardless of census. They are hired, credentialed, trained, and retained — and their salaries are paid every two weeks whether the branch has 20 patients or 80. As census grows, the clinical team grows with it — by requirement, not by choice. A new RN Case Manager is required for every 12 patients on service. A Hospice Aide for every 8.

Clinical Role	What They Do for Each Patient	Annual Cost
Executive Director	Leads clinical operations, regulatory compliance, and team performance. Accountable for the standard of care every patient receives — and the first person responsible when that standard is not met.	\$140,000
Supervisor RN Case Manager	Manages the clinical staff and ensures care quality is consistent across every patient. Bridges the gap between the office and the bedside. Catches problems before they reach patients.	\$110,000
RN Case Manager (×2 min; +1 per 12 patients)	The primary clinical contact for each patient. Visits at home twice a week or more. Manages pain, assesses symptoms, adjusts medications, coordinates care, and — over weeks or months — becomes one of the most trusted people in a dying patient's life outside of their family.	\$100,000 each
Hospice Aide (×2 min; +1 per 8 patients)	Provides personal care — bathing, hygiene, comfort, and the preservation of human dignity. Often the team member with the most frequent contact. Provides hands-on care that keeps patients clean, comfortable, and connected to their sense of self in their final weeks.	\$50,000 each
Social Worker	Carries the weight that the family has been carrying alone. Navigates financial resources, insurance, facility communication, and family dynamics. Helps a daughter stop being her father's medical coordinator and start being his daughter again. Grief support, care planning, and the invisible work of holding families together.	\$75,000

Chaplain	Provides care for the dimension of dying that medicine cannot reach. Shows up without an agenda and sits with whatever a patient needs to say — fear, regret, faith, doubt, unfinished business, or silence. For many patients, the chaplain is the person they remember most.	\$70,000
After Hours RN	The voice on the other end of the 2 a.m. phone call. When a patient's breathing changes and their family panics, this is the person who answers, assesses, guides, and — when necessary — goes. No patient faces a nighttime crisis without a trained clinician available. This position makes that possible.	\$95,000
Weekend RN	Ensures care does not pause on Saturday and Sunday. A patient's need for pain management, symptom control, or clinical support does not observe business hours. This role ensures the clinical team is never truly off — because neither is the work.	\$95,000
Medical Director (Contract)	Certifies eligibility, oversees symptom management protocols, and provides physician-level oversight of every patient's plan of care. Required by Medicare for every certified hospice. The clinical authority who ensures each patient receives medically appropriate, evidence-based comfort care.	\$75,000

As census grows, so does the clinical team — by Medicare requirement, not by choice.

An RN Case Manager is required for every 12 patients on service. A Hospice Aide for every 8. The branch cannot delay those hires without risking compliance — and, more importantly, without failing the patients already enrolled. When census falls, these are the last positions eliminated — because eliminating them means abandoning patients.

What happens in practice when census drops and the branch cannot sustain this team fully? Leadership delays replacing a departing RN. After-hours coverage gets stretched thin. An aide carries more patients than she should. The chaplain's hours get cut. Training budgets disappear. The clinical team feels the pressure even if they cannot name its source — and the patients enrolled feel it too, even if no one says so out loud.

"That is the human cost of insufficient census. It does not show up on a balance sheet. It shows up at the bedside."

The Operational Team — The Infrastructure Behind Every Visit

Behind every clinical visit is a referral processed, a schedule built, a supply ordered, a care plan documented, and a billing cycle completed. These roles exist so that the clinical team can remain focused entirely on patients — not paperwork.

Operational Role	What They Make Possible	Annual Cost
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Intake Coordinator	Receives referrals, processes admissions, and coordinates the first clinical visit. Often the first human voice a patient's family hears after the physician says hospice. That first interaction — its warmth, its speed, its competence — shapes the family's trust in the branch before a single clinician walks through the door.	\$60,000
Secretary / Administrator	Scheduling, documentation, billing support, and office operations. The operational foundation that keeps the branch functioning — without which clinicians spend their time on paperwork instead of patients, and the entire care system slows down.	\$55,000

The Sales Rep — The Person Who Makes All of This Possible

Every person on the list above — the nurse, the aide, the chaplain, the social worker, the coordinator answering the phone at midnight — has a job because someone built a census. That someone is you.

The hospice sales rep is not a support function. The rep is the economic engine that funds the mission. Every admission you generate is a patient who receives expert, compassionate, coordinated end-of-life care instead of a hospital room or a frightened family navigating a crisis alone. Every referral relationship you build and maintain is a pipeline of future patients who will receive that care.

You are not selling a Medicare benefit. You are connecting the people who need this team to the team that needs them.

PART TWO

The Daily Cost Reality

What it costs to care for each patient, every day

Revenue from Medicare's Routine Home Care rate arrives as a flat per-diem — the same daily amount whether the patient had a crisis visit last night or a routine aide visit. What many reps do not realize is that a significant portion of that per-diem is consumed before a single salary is paid. These are the direct, variable costs incurred for every patient, every day — costs that scale with census whether the branch is growing or contracting.

The Revenue Structure — Understanding the Per-Diem

Medicare pays two Routine Home Care rates based on how long a patient has been enrolled in the current benefit period. The 2025 national base rates are:

- **Day 1–60:** \$224.62 per patient per day — the higher rate for early-enrollment days
- **Day 61+:** \$177.27 per patient per day — approximately 21% lower for extended stays

These rates are adjusted by each region's wage index, so actual rates vary by market. The simulator allows you to enter your specific contracted rates. For planning purposes, most branches use the national base rate as a conservative benchmark.

A patient with a 30-day average length of stay generates revenue entirely at the higher Day 1–60 rate. A patient with a 120-day stay generates half their revenue at the lower Day 61+ rate. This means branches with higher average length of stay have a structurally lower blended revenue per day — which directly raises the census required to break even.

What the Per-Diem Pays For First — Variable Clinical Costs

Before payroll, before overhead, before any operating profit, the branch must fund the direct clinical costs of caring for each patient each day. These are not administrative costs — they are the actual medications, equipment, supplies, and travel expenses incurred at the bedside.

Cost Category	What It Covers	Typical Range / Day	Base Preset
Pharmacy	All medications prescribed as part of the hospice plan of care — pain management, anti-nausea, anxiety, comfort agents, wound care supplies. The single largest variable cost for most patients.	\$15 – \$60	\$22.00
DME (Durable Medical Equipment)	Hospital beds, wheelchairs, commodes, oxygen concentrators, and bedside equipment that keeps patients safe and comfortable at home without hospital-level infrastructure.	\$6 – \$15	\$9.00

Medical Supplies	Gloves, dressings, incontinence products, wound care materials, catheters, and the clinical supply bag that goes into every patient's home on every visit.	\$6 – \$20	\$8.00
Travel & Transportation	Mileage reimbursement and drive-time costs for every RN, aide, social worker, and chaplain visit. In rural or geographically spread territories, this line item alone can significantly affect profitability.	\$4 – \$12	\$7.00
Other Direct Patient Costs	Contracted therapy services, lab draws, interpreter services, and any per-patient expense not captured above.	\$2 – \$10	\$4.00
Total Variable Cost Per Patient Per Day	The baseline spend incurred the moment a patient is on service.	\$33 – \$117	\$50.00

Base preset reflects national average acuity. Oncology, wound-heavy, or complex pain management patients can reach \$80–\$120/day in variable costs alone.

Contribution Margin — What Remains After Direct Costs

After subtracting all direct variable costs from the blended RHC per-diem, what remains is the contribution margin per patient per day. This is the money available to pay for the people on the previous pages — the nurses, aides, social workers, chaplains, after-hours staff, administrators, and leadership that make every patient's experience what it is.

Blended Planning Assumption: \$120 – \$155 per patient per day

At national base RHC rates with standard variable costs and a typical length of stay, most hospice branches operate with a contribution margin between \$120 and \$155 per patient per day. The Spartan simulator calculates your exact figure based on your actual rates and cost assumptions. This single number drives every financial threshold in your branch's profitability model. Every patient admitted generates this amount toward the fixed costs that keep the clinical team employed. Every patient not admitted is that amount per day that does not exist.

It is worth pausing on that last point. When a patient who would have benefited from hospice care dies in a hospital instead of at home — because no one ever made the referral, or because the sales rep did not follow up — the branch does not just lose revenue. A family loses the RN who would have called at 2 a.m. A patient loses the aide who would have helped them bathe with dignity. A daughter loses the social worker who would have told her she didn't have to do this alone.

This is why the work of building and maintaining census is not a sales function disconnected from clinical care. It is the precondition for clinical care to exist.

PART THREE

The Numbers That Should Have Been Explained on Day One

The math behind break-even, target margin, and why your census goal is exactly what it is

Everything in the previous two sections leads to this: the financial math that connects a specific census target to the ability to maintain the clinical team in full. These formulas are not complicated. They are simply never explained to the people — hospice sales reps — whose daily work determines whether the numbers come true.

The Chain From Census to Profit — How the Math Works

1	Average Daily Census (ADC)	The number of patients actively on service on any given day. This is the starting point for every other calculation. Every additional patient on census adds their contribution margin to the pool that funds the clinical team.
2	Blended Revenue Per Day	$ADC \times \text{the blended RHC rate (adjusted for length of stay mix). For LOS} > 60 \text{ days: } ((\text{Day1Rate} \times 60) + (\text{Day61Rate} \times (\text{LOS} - 60))) \div \text{LOS}.$
3	Annual Revenue	$ADC \times \text{Blended Revenue Per Day} \times 365.$ The total Medicare revenue the branch generates at that census for a full year.
4	Annual Variable Cost	$ADC \times \text{Total Variable Cost Per Patient Per Day} \times 365.$ The direct clinical costs — pharmacy, DME, supplies, travel, other — that scale with census.
5	Annual Payroll	The sum of all staff salaries required at that census level, including the scaling roles (RN Case Managers, Hospice Aides) that must grow as census grows.
6	Annual Overhead	Monthly non-payroll fixed costs (office, EMR, insurance, compliance) $\times 12.$ These costs do not change with census — they are the same whether you have 10 or 100 patients.
7	Annual Profit / (Loss)	$\text{Revenue} - \text{Variable Cost} - \text{Payroll} - \text{Overhead}.$ The operating income available after all costs are covered. A negative number means the branch is losing money every single day at that census.
8	Operating Margin %	$\text{Annual Profit} \div \text{Annual Revenue} \times 100.$ The industry benchmark for a healthy hospice branch is 12–18%. Below 10% is considered fragile. Above 20% indicates a branch with the financial capacity to expand, invest in quality, and absorb disruptions.
9	Break-Even ADC	$\text{Annual Fixed Cost} \div (\text{Contribution Margin Per Day} \times 365).$ The census where revenue exactly covers all costs with no profit remaining. This is the survival floor — not the goal.

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0**Target Margin ADC**

$\text{Annual Fixed Cost} \div (365 \times (\text{Blended Revenue} \times (1 - \text{Target Margin\%}) - \text{Variable Cost}))$. The census where the branch hits its operating margin goal. This is the actual sales target — the number that closes the gap between survival and sustainability.

Break-Even ADC — The Floor, Not the Goal

Break-even is the census level at which the branch covers every cost — salaries, medications, equipment, overhead — with nothing remaining. At break-even, every nurse is paid, every medication is funded, every supply is stocked. What break-even cannot pay for: merit raises, clinical education, quality improvement programs, equipment upgrades, or any buffer against a sudden census drop. Break-even is survival, not sustainability.

Typical Break-Even Range: ADC 29 – 40 (Base Preset)

Using national base RHC rates, standard variable costs, and a lean but complete staffing model, most new hospice branches hit break-even somewhere between ADC 29 and ADC 40 — depending on overhead structure, local wage rates, and patient acuity. The Spartan simulator calculates your branch's exact break-even ADC based on your actual numbers. Below break-even, the branch is losing money on every patient-day. The clinical team is present and serving — but the business is drawing down capital that will eventually run out. Break-even is the minimum threshold for the clinical team to be fully funded. It is not a finish line.

Target Margin ADC — The Real Sales Goal

Every hospice branch operates with a target operating margin — typically 12–18% of annual revenue. This margin is not profit extracted from patient care. It is the financial cushion that allows the organization to replace aging equipment, retain experienced clinical staff, build out underserved markets, and survive a census disruption without immediately cutting the clinical team.

The Target Margin ADC is the census required to hit that margin goal. It is always higher than break-even — often by 15–25 patients. The gap between break-even and target margin ADC is the sales gap. It is the specific number of additional patients whose admissions move the branch from surviving to thriving.

"The gap between break-even and target margin ADC is the sales gap. That is the number your work closes. That is what your admission activity means."

Monthly Admissions — The Conversion From ADC to Activity

ADC is a stock — the count of patients currently on service. Admissions are the flow that maintains it. Every month, patients leave the census through death, discharge, or revocation. The number of new admissions required to replace that attrition and maintain target ADC is determined by a single formula:

Monthly Admissions Formula

Monthly Admissions Needed = $(\text{Target ADC} \times 365) \div \text{Average LOS (days)} \div 12$ Example: Target ADC 60, Average LOS 90 days = $(60 \times 365) \div 90 \div 12 = 21,900 \div 90 \div 12 = 20.3$ admissions per month ≈ 4.7 per week

Required Admissions by Target ADC — Assuming 90-Day Average LOS

Target ADC	Patient-Days / Year	Monthly Admits	Weekly Admits	Marketers Needed @ 8 admits/mo	Annual Revenue (blended ~\$195/day)
50	18,250	~17 / month	~4 / week	~3	~\$3.6M
60	21,900	~20 / month	~5 / week	~3	~\$4.3M
80	29,200	~27 / month	~6 / week	~4	~\$5.7M
100	36,500	~34 / month	~8 / week	~5	~\$7.1M

Assumes 90-day average LOS. Revenue estimate uses blended rate ~\$195/patient/day. Use the Spartan simulator with your actual rates for precise figures.

The ADC 60 Reference Point — 20 Admissions, 5 Per Week

A branch targeting ADC 60 with a 90-day average length of stay needs approximately 20 admissions per month — roughly 5 per week. That is not a quota a manager invented. It is the mathematical rate at which new patients must enter service to replace the patients who are dying and being discharged at the other end. Every missed admission is a gap in that flow. That gap does not show up as a line on a report. It shows up as a nurse who cannot be hired, an aide whose caseload grows beyond what is safe, a chaplain whose hours get cut — and eventually, a patient who does not receive the care they were promised.

PART FOUR

Reading the Field Differently

Now that you understand what the number means

Most hospice sales reps walk into a referral source's office thinking about their own number — how many admissions they need to hit their quota, what their manager will say if they come up short. The rep who has read this far walks in thinking about a different set of numbers — and that shift changes everything about the conversation.

What Changes When You Know This

You stop selling a benefit and start funding a team.

The difference between a branch at ADC 45 and ADC 65 is not an administrative metric. It is the difference between a branch that can afford its after-hours RN and one that cannot. Between a branch that can retain its most experienced case manager and one that loses her to a competitor with a more stable census. Every admission you generate is a contribution to that team's ability to function at full capacity.

Your referral source's hesitation costs someone a care team.

When a physician delays a hospice referral, or when a facility liaison is noncommittal about referring to your branch, the patient who would have benefited is the one who pays. Not in dollars — in care. The family that needed a social worker to show up. The patient who needed a chaplain. The family member who needed someone to tell her she didn't have to do this alone.

Your own follow-through is a clinical act.

The admission that didn't happen because you didn't follow up is a patient who didn't get enrolled. That is a clinical outcome, not a sales outcome. Reps who understand this don't need to be motivated to follow up. They feel the weight of not doing it.

Your activity targets now have a why.

Five admissions per week. Twenty per month. These are not arbitrary activity standards. They are the admission rate required to hold a census that sustains the clinical team. Every referral conversation you have either moves that rate forward or leaves the gap in place.

Break-even is not the destination. Sustainability is.

A branch at break-even is surviving. It can fund the team today. It cannot invest in them tomorrow. A branch at target margin can replace equipment, retain experienced staff, expand into underserved communities, and absorb the disruptions — the pandemic, the staffing crisis, the reimbursement change — that will inevitably come. The gap between your current ADC and target margin ADC is the gap between a branch that exists and one that grows.

"You are not closing a sale. You are connecting a patient to a team of people who have trained their entire careers for this specific moment in that patient's life."

What to Say When You Know This

You do not need to lead with profitability math in a referral conversation. You lead with the clinical team. But knowing the math gives you the authority to say what follows with conviction.

- "Our branch has the clinical depth to handle complex patients because our census allows us to staff for it. An RN Case Manager for every 12 patients. Dedicated after-hours coverage every night. That is not every branch."
- "The families who come to us trust us with the most important time of their lives. We take that seriously enough to make sure we have the staffing to do it right."
- "Every patient we enroll helps us maintain the team. Every patient we cannot reach is one fewer family with access to the level of care we can provide."

None of these statements are marketing language. They are true. They are grounded in the math and the mission this document has described. When you say them, you say them as someone who understands what they mean — and that comes through.

The Branch Profitability Simulator

A live model of your branch's financial reality — built on exact formulas

The Spartan Branch Profitability Simulator is an interactive tool that computes every number in this document for your specific branch, using your actual revenue rates, clinical costs, staffing structure, and operating assumptions. It is not a spreadsheet approximation. Every calculation uses Decimal.js precision arithmetic applied to the exact formulas described in Part Three.

What the Simulator Computes

Input	What You Enter	What It Generates
Census & LOS	Target ADC, Average Length of Stay	Blended revenue per day, monthly and weekly admissions needed
Revenue Rates	RHC Day 1–60, RHC Day 61+	Annual revenue at target ADC, exact blended rate for your patient mix
Variable Costs	Pharmacy, DME, Supplies, Travel, Other	Contribution margin per patient per day, annual variable cost
Fixed Overhead	Monthly non-payroll overhead	Annual fixed cost base, break-even ADC
Staffing	Auto-computed from ADC; fully editable	Annual payroll at target census, payroll scaling as ADC changes
Starting Capital	Available launch capital	18-month cash runway simulation with month-by-month P&L;
Sales Assumptions	Admissions per marketer per month	Marketer headcount required, individual weekly admission target

Five Ways to Use the Simulator in the Field

- 1. Frame the stakes before a difficult conversation.** Before a referral source meeting where you need to rebuild trust or recover lost volume, run the simulator at your current census. Know what the gap costs the branch per month. Know what the clinical team looks like at your target ADC versus your current ADC. Speak from numbers, not from urgency.
- 2. Translate your admission target into weekly activity.** Use the monthly admissions output to calculate your required close rate and weekly call cycle. If you need 5 admissions per week and your qualified-referral-to-admission conversion rate is 60%, you need 8–9 qualified referral conversations per week — not 15 cold calls. That drives your territory plan.

3. Show leadership what each census milestone unlocks. ADC 40 gets you to break-even. ADC 55 allows you to add a clinical FTE. ADC 70 creates the margin to invest in a quality program or a second territory. Use the simulator to build the business case for the resources your sales team needs.

4. Anchor referral conversations to the clinical team. The most powerful version of this conversation is not about your numbers — it is about the people your census funds. Share what it costs to keep an RN available at 2 a.m. Share what a fully staffed aide team means for the patients a facility refers. Census is not an administrative metric — it is the financial oxygen the clinical team breathes.

5. Model the cash runway for new branch planning. For reps involved in branch launches or expansion decisions, the 18-month runway simulation shows exactly how many months of starting capital the branch has before census must cross break-even. This gives the sales team a hard deadline and a clear ADC milestone to build their ramp around — before the capital runs out.

Access the Simulator

The Spartan Branch Profitability Simulator is available to all Spartan Coaching members at spartancoaching.com. Enter your actual revenue rates, clinical costs, and staffing assumptions to generate a customized profitability model for your branch. Every result is based on your real numbers — not national averages or generic benchmarks. Results can be exported as a PDF for team or leadership review.

What This All Means

The census is not the point. The care is.

Everything in this document has been in service of one idea: the census number you were given is not arbitrary. It is not a number someone invented to make your life harder. It is the precise minimum condition under which the most influential people in the dying process can do their jobs — fully, consistently, and with the clinical depth the patients in your territory deserve.

The RN Case Manager who becomes one of the most trusted people in a dying patient's life needs a caseload that allows her to actually know her patients — not manage a number. The hospice aide who preserves a man's dignity on one of his last mornings needs a supervisor who can cover her if she needs support. The social worker who finally tells a daughter that she does not have to do this alone needs an organization stable enough to have hired her and keep her.

All of that requires census. Not a specific number on an arbitrary target sheet. Census that is high enough to fund the team. Consistent enough to retain the team. Growing enough to allow the team to do more than survive the week in front of them.

"Every admission is a family who gets this team instead of navigating the end of life alone. Every gap in census is a team that cannot be fully present for the families who need them."

Your work — your referral conversations, your call cycles, your follow-through, your presence in the territory — is not a sales function layered on top of the clinical mission. It is the engine that makes the clinical mission economically possible. Without your census, there is no clinical team. Without a clinical team, there is no care. Without the care, the patients who entrusted your branch with the most sacred chapter of their lives receive something less than what they deserved.

That is what the census number means. That is why it is not arbitrary. That is why the work matters.

The census is not the point. The care is. The census is simply what makes the care possible.

KEY TERMS

Quick Reference Glossary

Average Daily Census (ADC)

The average number of patients on hospice service on any given day. The primary revenue driver for every branch — each additional patient generates \$120–\$155 per day in contribution margin toward fixed costs, payroll, and ultimately the ability to sustain the clinical team.

Average Length of Stay (LOS)

Average number of days a patient remains on service before death or discharge. Directly determines monthly admissions volume: shorter LOS means more admissions are needed to hold census. Longer LOS shifts revenue toward the lower Day 61+ rate but reduces the monthly admission requirement.

Blended Revenue Per Day

The weighted average RHC reimbursement rate accounting for the proportion of patient-days at the Day 1–60 rate versus the Day 61+ rate, based on average LOS. Formula (LOS > 60): $((\text{Day1Rate} \times 60) + (\text{Day61Rate} \times (\text{LOS} - 60))) \div \text{LOS}$.

Break-Even ADC

The minimum census at which total revenue exactly covers all costs — salaries, patient costs, and overhead. Below this number, the branch loses money every single day. Typically 29–40 patients for a lean standard community-based branch. This is the floor below which the clinical team cannot be fully sustained.

Cash Runway

How many months starting capital can sustain a branch operating below break-even before reserves are exhausted. Critical for new branch launch planning — the ramp timeline must reach break-even before runway runs out, or the branch will not survive long enough to fulfill its mission.

Contribution Margin

Revenue per patient day minus direct variable patient costs (pharmacy, DME, supplies, travel, other). Approximately \$120–\$155 per patient per day in a standard acuity branch. This is the amount each enrolled patient generates toward the fixed costs that keep the clinical team employed and the branch operational.

Monthly Admissions Needed

New patient enrollments required each month to maintain target ADC, calculated as $(\text{Target ADC} \times 365) \div \text{Avg. LOS} \div 12$. This number directly drives marketer headcount, individual production targets, and the entire sales plan.

Operating Margin

Annual profit as a percentage of annual revenue. A healthy hospice branch targets 12–18%. This margin is not extracted from patients — it is the financial cushion that allows the branch to invest in clinical staff, expand to underserved markets, and sustain the care team through disruptions.

Routine Home Care (RHC)

The Medicare per-diem rate paid for each day a patient is enrolled in hospice. Days 1–60 are reimbursed at a higher rate (\$224.62 at 2025 national base) than days 61 and beyond (\$177.27). The primary revenue mechanism for every hospice branch — a flat daily payment for each patient on service.

Target Margin ADC

The census needed to achieve the operating margin goal (typically 12–18%). This is the real sales target — not just survival but sustainability. The gap between break-even and target margin ADC is usually 15–30 patients, and it represents the difference between an organization that survives and one that can grow and invest. Formula: $\text{Annual Fixed Cost} \div (365 \times (\text{Revenue} \times (1 - \text{Target\%}) - \text{Variable Cost}))$.

Weekly Admissions Needed

$\text{Monthly Admissions Needed} \times 12 \div 52$. The weekly admission rate required to maintain target ADC. This is the operational target that drives call cycles, territory plans, and marketer headcount requirements.

Access the Branch Profitability Simulator and full field education library at spartancoaching.com

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Every input is customizable. Every output is actionable. Start with your real numbers.